Breast Examination — OSCE Checklist

Deakin MD2 · ELC7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Introduction → Inspection (seated) → Axillae + supraclavicular (seated) → Breast palpation (supine) → Close**. *Examine* **both breasts and draining nodes**. *Communicate throughout in your own words.*

1. Introduction (intimate examination)

☐	**Introduce** self (name, role); confirm patient ID
☐	**Explain** what the exam involves and why; check understanding
☐	**Offer a chaperone / observer** (document if declined)
☐	**Consent**: verbal, informed; "Just say stop and I will"
☐	**Wash hands**; gloves where indicated
☐	**Privacy**: undress behind curtain, drape/cover, warm room, good light
☐	Ask about symptoms, noticed changes, past surgery or radiotherapy
☐	Position: seated on the couch; drape so one breast/axilla is exposed while the other is covered

2. Inspection — seated, arms by sides

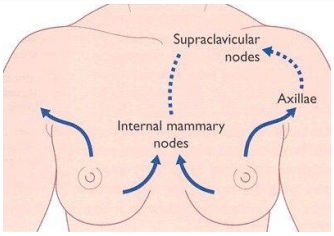
	Inspect for	What to say / significance
☐	Introduce step	"I'm going to start by looking at your breasts. Could you sit up straight and lower your gown when you're ready?"
☐	Size, shape, symmetry	Asymmetry may be normal — compare with patient's baseline
☐	Lumps or thickenings	Visible contour change
☐	Skin puckering / dimpling	Tethering by underlying tumour
☐	Peau d'orange	Pitted oedematous skin — lymphatic obstruction
☐	Erythema, prominent veins	Inflammation, infection, inflammatory carcinoma
☐	Sores or ulcers	On breast or nipple
☐	Nipple retraction	New drawing-in of the nipple
☐	Rash on / around nipple	Eczematous change — think Paget's disease of the nipple
☐	Axilla and upper arm	Axillary swelling or lumps; arm swelling (lymphoedema)
☐	Pendulous breasts*	Ask patient to lift breasts to inspect underneath

3. Inspection with movement

	Manoeuvre	Look for
☐	"Could you follow my arm movements?" — raise arms above head	Tethering: tissue, skin or nipple not moving smoothly or symmetrically
☐	Hands on hips, lean forward and press hands into hips (pectoral contraction)	Increased dimpling = fixation to chest wall

4. Palpation — lymph nodes (seated)

☐	Hold the patient's outstretched arm with yours to relax the axilla; palpate with your free hand (gloves may be preferred)
☐	Palpate all 5 walls: **anterior, posterior, medial, lateral, apex**
☐	Repeat on the other axilla — always examine both
☐	Palpate supraclavicular fossae both sides
☐	"Thank you. Pull the gown up, and when you're ready, please lie down."

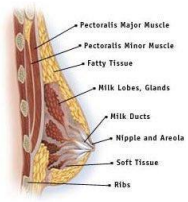


Lymph drainage of the breast

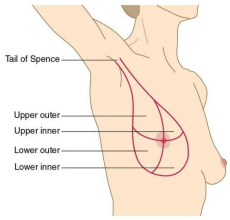
5. Palpation — breasts (supine)

☐	Uncover **one breast at a time**; patient's arm abducted with hand behind head
☐	Flat pulps of fingers, small circles: light pressure then deeper, before moving on (other hand may support tissue)
☐	Cover the whole area: clavicle → sternum → mid-axillary line → bra line
☐	Systematic pattern: vertical strips ("lawn mowing") or spiral out from the nipple; minimise lifting hand off skin
☐	Palpate behind the nipple for lumps
☐	Include the **axillary tail** (superolateral)
☐	Nipple discharge*: ask patient to express if discharge reported; note colour, single/multiple ducts †

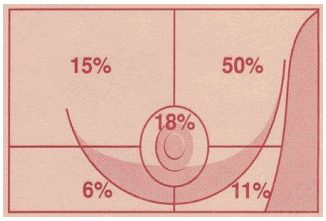
☐	Repeat fully on the other breast



Breast anatomy



Breast quadrants and axillary tail



Where breast cancers most commonly occur

6. If you find a lump — describe it

☐	Position	Quadrant, clock-face position and distance from areola
☐	Size	Diameter in cm (or pea / grape / plum)
☐	Shape and surface	Rounded, regular / irregular, smooth / rough
☐	Consistency	Fluctuant, soft, firm, hard
☐	Tenderness	How tender
☐	Mobility	Mobile or fixed — to skin or deep (chest wall)
☐	Borders	Discrete or poorly defined
☐	Retraction	Altered contour, dimpling
☐	Number	Single or multiple; check the other breast

7. Closing

☐	Thank patient; let them know they can dress; give privacy
☐	Explain findings and next steps; invite questions
☐	Document consent, chaperone, examination, findings and discussion
☐	Present: "On inspection the breasts were symmetrical with no skin or nipple changes. No masses on palpation of either breast or the axillary tails, and no axillary or supraclavicular lymphadenopathy. This is a normal breast examination."

8. Quick reference

Feature	Fibroadenoma †	Cyst †	Carcinoma †
Age	15–35	35–55, perimenopausal	Usually >40
Feel	Smooth, rubbery	Smooth, fluctuant / firm	Hard, irregular
Mobility	Very mobile ("breast mouse")	Mobile	Often fixed
Skin / nipple	Normal	Normal	Dimpling, peau d'orange, retraction

Key fact	From the Deakin guideline
Triple test	Clinical exam + imaging (mammogram/US; MRI gold standard) + non-excisional biopsy for **any** lump
BreastScreen	Free mammogram 2-yearly from 40; actively invited 50–74
Burden	Most common cancer in Australian females; 1 in 8 by age 85
Lymph drainage	Lateral/central → axillary nodes; medial → internal mammary (not palpable)
Commonest site	Upper outer quadrant †

- Breast examination alone is **not** recommended as a screening test (RACGP) — use it to teach breast awareness.
- Report promptly: new lump, nipple change or discharge, skin colour/texture change, pain, swelling.
- Pitfalls: exposing both breasts at once, missing the axillary tail or behind the nipple, forgetting supraclavicular nodes, no chaperone offer.